

Adrabetadex: U.S. Reimbursement Landscape, HCPCS, and Pass-Through

Launch friction, coding, and outpatient payment brief

Audience: Client-facing access and commercialization stakeholders

Prepared March 30, 2026. Facts reflect the attached source set and public CMS, HRSA, Medicaid, Medicare, and payer policy sources reviewed on March 30, 2026.

Launch reality in one sentence

Adrabetadex enters a market where the medical benefit supports coverage in principle, but coverage is operationalized through prior authorization, specialist prescribing, combination restrictions, white-bagging rules, site-of-care review, and claim-level documentation. Launch success therefore depends on coding and payment execution as much as on clinical differentiation.

The internal source set recognizes this indirectly by assuming favorable buy-and-bill site economics and a modest 15% gross-to-net burden. The aligned provider model now carries a working WAC of \$39,000 per 900 mg vial, commercial reference-price views at 15%, 20%, and 30% below WAC, and non-340B manufacturer-support sensitivities at 5% and 10% because the provider model shows that commercial margin remains thin without either 340B access or additional support. The public record on payer management shows a controlled environment in which plans already manage analogous NPC and rare neurologic therapies with genetic confirmation, functional continuation criteria, sourcing rules, and site-of-care restrictions.

What commercial plans already do

- UnitedHealthcare requires Miplyffa to be used with miglustat, prohibits concurrent Aqneursa, and requires documented positive clinical response for reauthorization.
- UnitedHealthcare requires Aqneursa to be used with miglustat or after failure, contraindication, or intolerance to miglustat, prohibits concurrent Miplyffa, and requires documented positive clinical response for reauthorization.
- UnitedHealthcare's medication sourcing protocol states that when a listed drug is sourced through an indicated specialty pharmacy, the pharmacy bills the plan directly and the outpatient provider may only bill for administration. If the provider does not obtain the drug through the indicated pharmacy, the plan denies payment for the drug.

- UnitedHealthcare's site-of-care policy states that non-hospital infusion sites, physician offices, ambulatory infusion suites, and home infusion are well accepted places of service, and hospital outpatient administration requires medical-record support when the policy applies.
- Aetna and other large plans use the same playbook in analogous therapies: baseline assessment plus continuation rules tied to measurable clinical benefit.

HCPCS background that matters at launch

HCPCS is the operational bridge between FDA approval and paid claims. CMS maintains the national Level II HCPCS code set for drugs, biologicals, and related items not described elsewhere. For hospital outpatient billing, C codes are the temporary HCPCS Level II codes used for drugs, biologicals, devices, and radiopharmaceuticals that receive OPSS pass-through status.

CMS now requires OPSS drug and biological pass-through applications and New Technology APC applications to be submitted through MEARIS. The CMS pass-through page states that only applications submitted through MEARIS will be accepted. That matters because adrabetadex will need a deliberate outpatient coding path rather than ad hoc local billing workarounds.

If a hospital outpatient department is billing an FDA-approved drug that does not yet have a product-specific HCPCS code and does not yet have pass-through status, CMS created code C9399 for that exact gap. CMS's hospital outpatient payment guidance also states that OPSS drugs without an assigned HCPCS code are paid at 95% of AWP while a permanent code gap remains.

Pass-through and New Technology APC: what they do and what they do not do

Transitional pass-through payment is temporary additional outpatient payment for new drugs, biologicals, and devices while CMS gathers cost data. CMS states that pass-through payment runs for at least 2 years but not more than 3 years. For drugs and biologicals, eligibility requires that the product be new to the outpatient payment system and that its cost be not insignificant relative to the related OPSS service payment.

CMS reviews pass-through applications quarterly. Under current policy, the drug pass-through payment amount is generally ASP plus 6% minus the portion of the APC payment amount that CMS associates with the drug or biological. In other words, pass-through is an incremental outpatient payment mechanism; it is not a blank additional payment layered on top of every other amount.

CMS's current quarterly MEARIS calendar means timing is concrete. A complete application filed by the first business day of March, June, September, or December can take effect on July 1, October 1, January 1, or April 1, respectively. Using the current August 17, 2026 PDUFA date, the earliest practical pass-through effective date is January 1, 2027 if the approval letter, final label, and pricing package are complete in time for the first business day of September 2026.

CMS also makes the data package concrete. The application needs the FDA approval letter, final label, trade and generic name, proposed HCPCS descriptor, dosage form, package size, method of administration, billing-unit logic, WAC, AWP, ASP if available, actual hospital cost, market-availability date, projected

utilization by site of care, and the requested HCPCS code or descriptor.

A New Technology APC is different. It applies to a truly new outpatient service that does not fit an existing clinical APC and is not eligible for pass-through. CMS places the service into a cost band and later reassigns it once sufficient claims data are available, generally after two to three years. For adrabetadex, the practical first question is whether the outpatient launch need is better described as a drug pass-through question, a service question, or both.

Why Medicaid and 340B still sit inside the outpatient coding story

For a pediatric launch, Medicaid is not a side topic. It is the core access channel in the internal payer mix. EPSDT means Early and Periodic Screening, Diagnostic, and Treatment. It gives the product a strong medical-necessity footing in beneficiaries under age 21, but it is a coverage rule rather than a single national reimbursement formula. Current official state examples show both ASP-based and WAC-based physician-administered drug payment. Colorado pays drugs in the Medicare ASP file at ASP minus 3.3% and pays drugs outside that file at WAC, while Pennsylvania pays brand physician-administered drugs at the lower of usual and customary charge or WAC plus 3.2%. Medicaid claim payment still depends on physician-administered drug data integrity, and Medicaid's PAD rules tie payment availability and federal matching funds to collection and submission of utilization data and, for defined categories, NDC information.

340B overlaps with that same claim pathway, and the economics are concrete. HRSA's rule is that a manufacturer may not charge a covered entity more than the 340B ceiling price whether the purchase runs through a wholesaler or directly from the manufacturer. The ceiling price equals AMP minus the Unit Rebate Amount. The minimum rebate percentages used in the current framework are 23.1% for most brand drugs, 17.1% for qualifying pediatric brand drugs and clotting factor products, and 13.0% for generics. On the working adrabetadex WAC of \$39,000, the new-drug estimate is \$29,991 at 23.1% and \$32,331 at 17.1%.

HRSA's duplicate-discount rule applies to Medicaid fee-for-service claims, requires carve-in or carve-out discipline, and publishes the covered entity's fee-for-service billing status in the Medicaid Exclusion File. That means outpatient coding, 340B inventory designation, and Medicaid billing cannot be managed as separate workstreams.

For dual-eligible beneficiaries, Medicare pays first for Medicare-covered services and Medicaid pays last. In the initial pediatric launch, dual eligibility will be rare. The operational rule still matters because any dual-eligible outpatient claim must be built as a Medicare-primary workflow with Medicaid wraparound rather than as a Medicaid-primary workflow.

Practical launch interpretation

The immediate implication for the client is straightforward. Commercial coverage criteria will exist, but those criteria will sit on top of a highly managed outpatient claims environment. A clean launch requires a product-specific coding strategy, a defined hospital outpatient interim-billing plan, a pass-through application decision, Medicaid NDC readiness, and 340B duplicate-discount controls that are live before the first broad commercial claims submission.

The aligned provider model clarifies what those mechanics mean economically. Commercial buy-and-bill at 100% of WAC produces zero drug spread, commercial white-bagging removes drug spread entirely, and the modeled 340B scenario produces a materially better spread because acquisition cost drops while the benchmark stays linked to the outpatient payment file. Under the aligned demo-model inputs, non-340B commercial margin only turns positive when additional manufacturer support reaches 5% or 10% of WAC, which is \$1,950 or \$3,900 per dose.

The source set already supplies the pricing and access assumptions. The public CMS, HRSA, Medicaid, and payer record supplies the mechanics. Taken together, they support a definitive launch view: adrabetadex reimbursement will be won or lost on operational execution, not on unmet need alone.

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